

Vertex AI Search Engine Demo

Show me evidence that Coley's Toxins actually cured or led to long term regressions in patients



Summary

Coley's Toxins have been reported to lead to long-term regressions in patients with sarcoma in the head or neck, as evidenced by several case reports. [1] ****Case Reports:**** ****Case 67 (BURKE):**** A 19-year-old female, H.R., presented with terminal sarcoma of the neck, face, and submaxillary region, involving bones, subcutaneous tissues, especially the forehead, parietal regions, left cheek, and jaw. [1] The onset was in March 1931, with a hard nodular mass in the neck below the angle of the right jaw. [1] Prior treatments included incision and x-ray therapy, which provided no relief, and a tracheotomy for edema of the glottis. [1] Radium was also given. [1] After treatment with toxins, the tumor disappeared in 5 to 6 weeks. [1] There was evidence of recurrence, and toxins were resumed and given with intervals of rest for 6 years. [1] She experienced complete regression and no further recurrence. [1] She married and had a child. [1] She was alive and well in 1945, 16 years traced after the onset of her condition. [1] ****Case 58 (DONALDSON):**** A female, aged about 20, had recurrent inoperable sarcoma of the scalp. [1] The specific details of the toxin treatment and outcome are not fully provided in the excerpt, but it is listed as an "Inoperable Success". [1] ****Case 59 (MACLEAN):**** A 2-year-old male presented with recurrent, rapidly growing angiosarcoma of the right cheek. [1] He was treated with toxins for 2.5 months, injected directly into the tumor, in 1918. [1] He experienced little febrile reaction, and complete regression occurred in a month, with no further recurrence. [1] He was alive and well in 1943, 25 years traced after the onset. [1] ****Strength of Evidence:**** The evidence presented consists of individual case reports, which are part of a document titled "SERIES A, INOPERABLE SUCCESSES". [1] These cases detail instances of

successful treatment of various inoperable cancers, primarily using "toxins," identified as commercial preparations of Coley's toxins available from 1907 to 1950. [1] Each case provides patient demographics, cancer type, treatment details, and long-term outcomes, including the "Years Traced After Onset". [1] The follow-up periods are extensive, ranging from 6 to 52 years, indicating long-term success in many instances. [1] Some cases involved prior surgical interventions or other treatments (like x-ray or radium) that were ineffective or incomplete before toxin therapy. [1] While these are compelling individual accounts, case reports generally represent a lower level of evidence compared to controlled clinical trials. However, the long-term follow-up in these cases (e.g., 16 years for Case 67 and 25 years for Case 59) strengthens the observation of sustained remission. [1]

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Snippets

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Extractive Segments

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relevance score: 0.82

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Snippets

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Extractive Segments

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relevance score: 0.62

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[from: coley-toxins-nauts-1953-long](#)

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Snippets

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Extractive Segments

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relevance score: 0.69

DISCUSSION It now seems opportune to review the evidence which has been recently assembled and to re-discover the effectiveness of Coley's original concept. With more precise knowledge of bacterial chemistry, of immune reactions, of antibiotics, of the defense mechanisms of the body, including inflammation and the alarm reaction, it may now be possible to determine the basic biological factors underlying the remarkable effects which were achieved by Coley and others who were working more or less empirically. In order to evaluate more clearly the actual results obtained by Coley's toxins and other forms of toxin therapy in cancer, we are now preparing a series of end-result studies of all known cases in which sufficient detail was available, and diagnosis was confirmed histologically. For a more detailed history of the early development of Coley's method, you are referred to the first two studies in this series (78, 79). Another more recent publication is a bibliography of approximately 500 references to the literature on the effects of actual infections, inflammation, fever or heat on malignant disease (80). We are also preparing a series of end-result studies according to anatomical sites. The first study comprises abdominal cancer. These cases have been sub-divided according to the organ in which the neoplasm developed wherever possible, and these in turn have been listed according to whether the patients were in the terminal, inoperable or operable stage when toxins were begun. The present monograph is a critical analysis of the method Coley developed with end-result studies of 30 inoperable cases treated by Coley's toxins. These cases indicate how the toxins worked on a variety of neoplasms, and includes cases showing the dangers of inadequate treatment. (Cases 22 and 27.) This group was selected from over 270 in which a complete regression was obtained, as being representative of what

could be accomplished by toxin therapy in inoperable malignant disease. Because the late Dr William B. Coley was put in charge of the Bone Tumor Service at Memorial Hospital, his practice after 1918 was largely devoted to bone cases, and therefore almost all his published papers in the last 15 years of his life deal with neoplasms of bone (34, 35, 36, 38, 43). Unfortunately, this gave the medical profession the erroneous impression that Coley's toxins were effective only in bone sarcoma. Actually, osteogenic sarcoma are very much less sensitive to toxins than neoplasms arising in the soft tissues. Coley stated this as early as 12 1895, but it was ignored. It is important to note that the osteolytic types of rapidly growing malignant bone tumors, such as endothelioma, reticulum cell sarcoma and plasma cell myeloma, as well as giant tumors of bone are all markedly sensitive to toxins. (See Cases 6, 8, 19, and 29 for examples.)

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Snippets

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Extractive Segments

page

relevance score: 0.84

from: The Toxins of William B Coley and the treatment of bone and soft tissue sarcomas

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Snippets

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Extractive Segments

page

relevance score: 0.57

Despite the downward spiral of Coley's treatment ideas, they never completely died. He himself remained undeterred, holding on to his belief in his toxins until the end of his career in 1933. He was not alone. In fact, by the early 1930s, a few doctors had changed their minds and were willing to accept that the toxins might be beneficial. In 1934, The Journal of the American Medical Association reversed its position and

agreed that Coley's Toxin might be of value: It appears, that undoubtedly the combined toxins of erysipelas and prodigiosus may sometimes play a significant role in preventing or retarding malignant recurrence or metastases; occasionally they may be curative in hopelessly inoperable neoplasms; . . . The Council has, for these reasons, retained Erysipelas and Prodigiosus Toxins-Coley in New and Nonofficial Remedies, with a view to facilitating further studies with the product.¹⁵ In a symposium held in 1935, Codman, apparently seeing evidence of the toxin's benefits, reversed his position and suggested that Coley's treatment might have some value after all. ¹⁶ Also, a controlled study done in 1962 showed a dramatic response in 20 of 93 cancer patients. ¹⁷ Further acceptance of his ideas was brought about by Coley's own children. His son Bradley (1892- 1961), also an orthopaedic surgeon, succeeded him as the head of the Bone Tumor Service at Memorial Hospital. Bradley Coley's major textbook on bone tumors was published in 1948, and while advocating surgery as the main treatment for bone sarcomas, he supported the use of Coley's toxin as adjunctive therapy. ¹⁸ He believed that it would be of value in preventing micro-metastasis. His daughter, Helen Coley Nauts (1907-2001), became a cancer researcher and devoted her life to the study of her father's toxins. She tabulated every patient he treated and reviewed all his notes. She published 18 monographs and tabulated over 1000 of his cases and noticed that in 500 of these there was near-complete regression.¹⁹ Nowadays, orthopaedic oncologists do not use Coley's Toxins for the treatment of bone and soft-tissue sarcomas. However, because many of these tumors are lethal, treatment options may one day be supplemented by immunotherapy. Since Coley's death, the field of immunology has developed into a sophisticated specialty. Scientists are studying the effect on tumors of such factors as tumor necrosis factor (TNF), interferons, streptokinase and many other cytokines, all related to the immune system. ²⁰ Indeed, vaccines are being developed for the treatment of numerous types of cancer, particularly colon cancer and melanoma.²¹ One form of immunotherapy which is consistently effective is the installation of BCG bacilli into the bladder to treat superficial bladder cancer.

from: THE EFFICACY OF COLEYS TOXIN IN THE TREATMENT OF SARCOMA

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Extractive Segments

page

relevance score: 0.70

Discussion. In the foregoing observations no evidence was obtained to indicate that Coley's toxins, or the other bacterial suspensions, inhibited the growth of sarcomata induced from the animals' own tissues by carcinogenic hydrocarbons. The acute hyperemia and hemorrhagic exudations that did occur, presumably due to such injections, did not result in appreciable growth restraint. As far as can be determined, the only other observations of this type reported upon sarcomata induced from the animals' own tissues by hydro- carbons are those of Andervont. This investigator found that B. coli filtrates "produced hemorrhage with regularity in primary 1:2:5:6 dibenzanthracene tumors," but that "thus far, complete recession of primary dibenzanthracene sarcomata has not occurred." The mechanism of the "hemorrhagic reaction" observed in some of the cases remains obscure. The assumption that it is an "immunologic phenome- non" also necessitates the assumption that the tumor tissue was primarily hypersensitive to certain bacterial products, a fact not yet demonstrated. The work of Shear and Andervont in obtaining a fraction from B. coli filtrates which exhibited a high potency as regards hemorrhage production in trans- plantable tumors would suggest that bacteria elaborate, in some form or an- other, a substance highly specific for the vascular system of tumors. Experi- ment VI was performed to observe whether in an acute inflammatory focus the

capillaries are in a hypersensitive state in regard to bacterial products. It was felt that a tumor presenting in a substantial part of its mass inflammatory reaction due to spontaneous necroses might thus prove to be hypersensitive to bacterial products. The results of this experiment, however, were negative. Furthermore, in the observations made above it would appear that the hyperemic and hemorrhagic reaction when once induced are not ephemeral phenomena but persist for some time. ### SUMMARY (1) The effects of intraperitoneal injections of "Coley's toxins," of killed 7-day dextrose broth cultures of *Streptococcus erysipielatis*, of killed 7-day dextrose broth cultures *B. prodigiosus*, and of killed 7-day broth cultures of *B. coli* were studied upon sarcomata of the subcutaneous tissues of rats, induced from the animals' own tissues by carcinogenic hydrocarbons. Transplanted neoplasms were not employed. (2) No evidence of inhibition of growth of the tumors was obtained. (3) In a significant number of instances the tumors of injected animals exhibited a marked hyperemic and hemorrhagic reaction. It is assumed that this was a reaction to the injection of bacterial products since such changes were not observed in control animals, and were definitely of a different type, macroscopically, than the spontaneous degenerative changes sometimes observed in such neoplasms. These reactions, as indicated, did not appear to markedly restrain the growth of the neoplasm.

from: SAM_0919

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Snippets

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Extractive Segments

page

relevance score: 0.79

from: SAM_0889

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Snippets

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Extractive Segments

from: Coley's toxins tumor necrosis factor and cancer research A historical perspective

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Snippets

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Extractive Segments

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relevance score: 0.73

8. SUMMARY AND CONCLUSIONS The permanent regression of inoperable sarcoma under the influence of erysipelas was a controversial observation even in Coley's time, stemming at least partially from the fact that sarcoma itself was not common (Coley, 1909), let alone sarcoma complicated by the development of erysipelas. Even when both of these conditions occurred simultaneously in the same patient, there was no guarantee that a permanent regression of the sarcoma would result, or even a therapeutic effect at all. With absolutely no understanding of the mechanisms involved and the

relatively limited percentage of even properly selected patients who were cured by the vaccine, Coley's efforts to gain widespread general acceptance of his treatment ultimately failed, the legitimacy of the original observations were questioned and the treatment itself remained controversial. Today, over a 100 years from the time Coley began his career, we have managed to piece together a reasonably distinct understanding of at least some of the mechanisms responsible for the phenomena that Coley and his contemporaries observed. An absolute lack of such understanding contributed much to the # Coley's toxins, tumor necrosis factor and cancer research 555 original controversy surrounding the treatment known as Coley's toxins. Today, with far less justification, the treatment has still yet to receive even the academic, let alone clinical attention that it deserves. Coley today is revered as the "father of present day immunotherapy" and awards are periodically presented in his name, most of the time for accomplishments, which while very important in and of their own right, yet at best, share only a peripheral association with Coley's work as a clinician. At the same time, in our current clinical efforts with recombinant cytokines, we find decisions being made empirically, by trial and error, that could have well been made in advance, had we expressed a true appreciation of Coley's work by simply reading his papers. Had we chosen to do so, we would have also learned that there is much more to be expected out of this area than that which we have managed to accomplish thus far. In view of recent observations regarding immune surveillance, it is unlikely that Coley's treatment was taking advantage of a system that was designed by evolution to do what he was sometimes able to make it do, i.e. cure inoperable sarcoma. It would not be surprising to find that the adverse effects of the treatment may well be directly linked to the therapeutic effects, and therefore, unavoidable. Furthermore, it is entirely possible that what Coley accomplished for his patients with the use of his vaccine was the best that anyone could ever do, even today. A more complete and thorough understanding of the problem of cancer does not necessarily guarantee us a more effective method of treating it.

[from: johnson-novales-coley's-toxins-1962](#)

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Snippets

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Extractive Segments

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relevance score: 0.77

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